Memorial General Hospital - Department of Inpatient Medicine

CLINICAL DOCUMENTATION — NARRATIVE INPATIENT DISCHARGE SUMMARY V1

Patient: James Lopez | **MRN:** PAT-60001 | **DOB:** 1965-04-05 | **Gender:** Male | **Doc ID:** NARRATIVE_0001
Encounter Date: 2025-02-01 | **Discharge/Note Date:** 2025-02-03 | **Provider:** Dr. Lisa Cuddy, MD

CHIEF COMPLAINT

The patient presented with worsening bilateral lower extremity edema and dyspnea on exertion.

HISTORY OF PRESENT ILLNESS

The patient is a 62-year-old male with a medical history pertinent for hypertension and dyslipidemia who presented for evaluation of worsening bilateral lower extremity edema and dyspnea on exertion. Symptoms began approximately four days prior to admission and escalated in severity. On presentation to the emergency department, initial diagnostic evaluation demonstrated that hemoglobin was 14 g/dL. Concurrently, serum chemistry was notable for a bun of 29.3 mg/dL (HIGH). Vital signs upon arrival were stable, and the patient was admitted to the inpatient general medicine service.

HOSPITAL COURSE

During the hospital course, the inpatient team initiated targeted management. The patient received Amoxicillin 250 mg orally every 8 hours for 7 days with good clinical tolerance. Serial assessments revealed gradual resolution of presenting symptoms. Prior to planned discharge, repeat laboratory testing confirmed that glucose stabilized at 87.9 mg/dL. The patient was ambulating without assistance and tolerating a regular diet.

DISCHARGE MEDICATIONS

The patient was discharged home in stable clinical condition with instructions to take Lisinopril 40 mg orally every morning, and Atorvastatin 10 mg by mouth every evening. Medication reconciliation was completed and all questions regarding dosing schedules were answered.

DISCHARGE INSTRUCTIONS & FOLLOW-UP

The patient was advised to follow up with their primary care physician within 7 to 10 days. Return precautions for chest pain, recurrent shortness of breath, or fever were provided and understood.